COVID-19 VAERS Reporting Pattern Analysis 2020–2026

Version 1.0
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Executive Summary

This project analyzed COVID-related VAERS reports from 2020–2026 including domestic and non-domestic records. The analysis utilized DuckDB, Python, confidence interval analysis, logistic regression, symptom enrichment analysis, manufacturer comparisons, lot analysis, and time-series methods. Key findings included: • Elevated myocarditis and pericarditis reporting among Pfizer-associated reports. • Elevated thrombosis, DVT, pulmonary embolism, CVST, Guillain-Barré, syncope, and loss-of-consciousness reporting among Janssen-associated reports. These findings remained present after age and sex adjustment and within a 2021-only rollout cohort. The analysis did not identify evidence supporting placebo lots, three-tier formulations, geographic targeting, or intentional lot deployment patterns.

Figure 1. Age- and sex-adjusted logistic regression odds ratios.

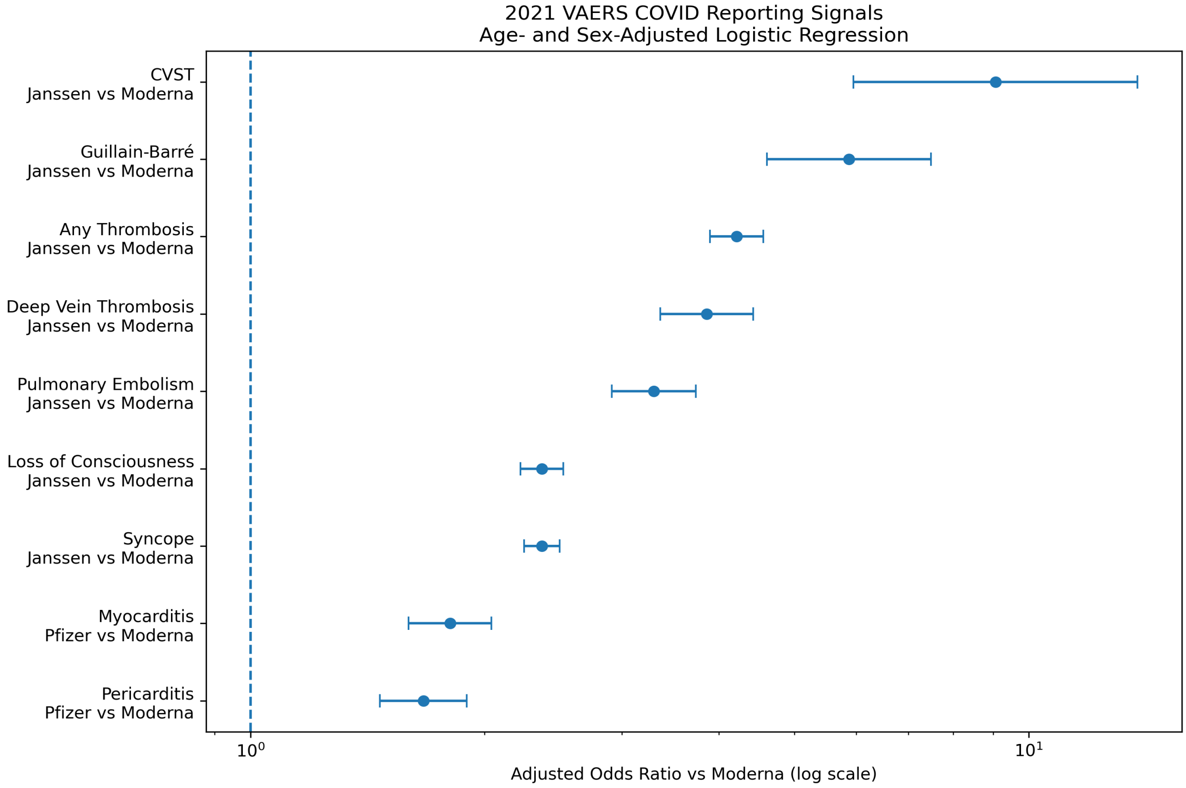


Figure 2. Male myocarditis reporting percentages by age group.

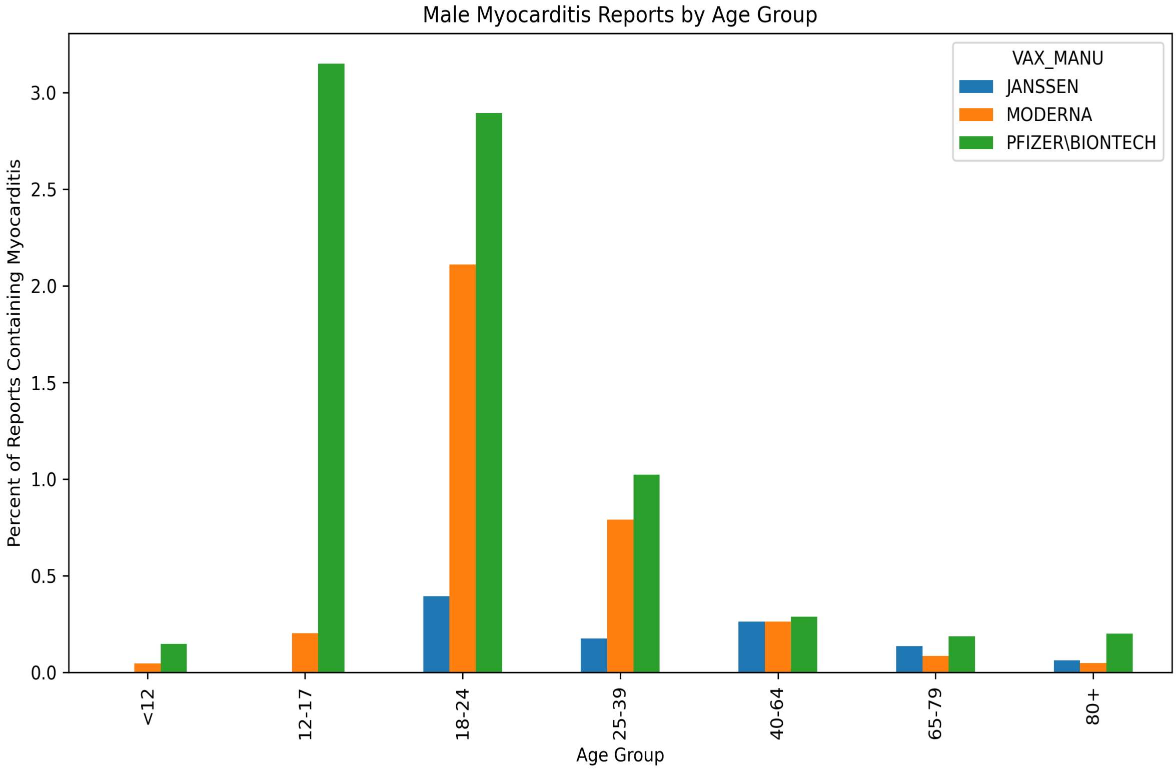


Figure 3. Clotting-related symptom terms by manufacturer.

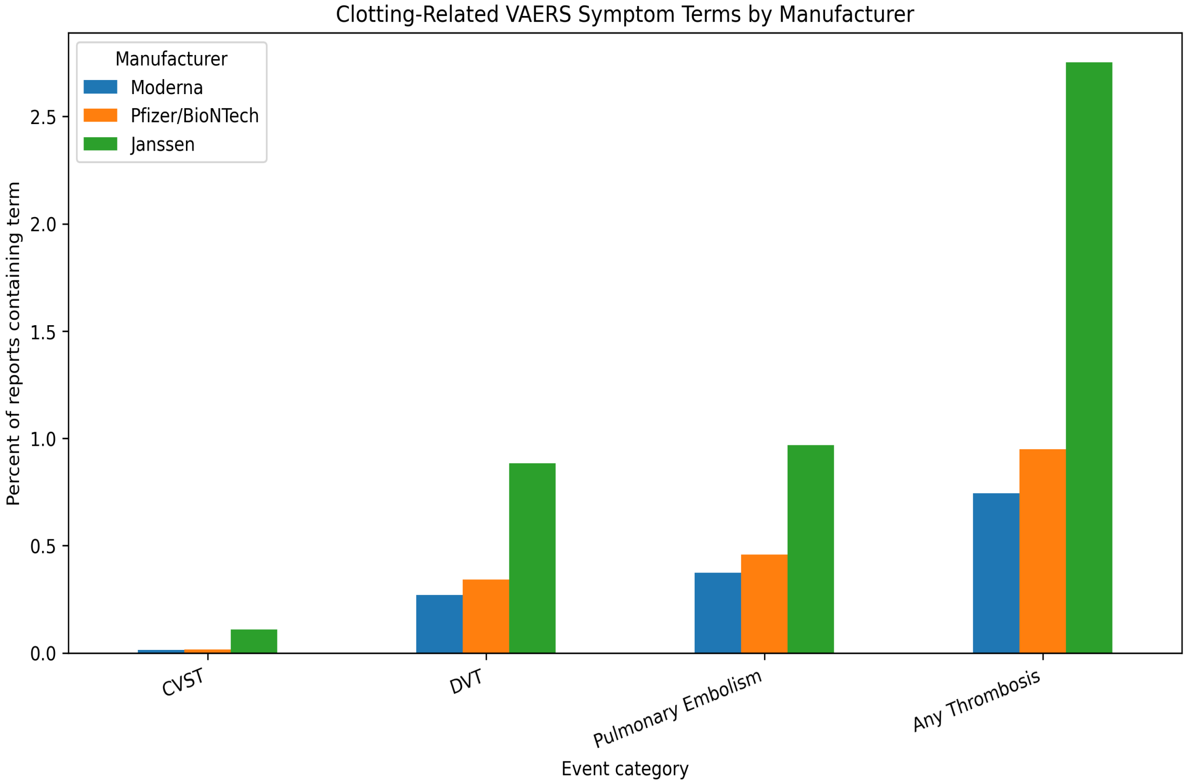


Figure 4. Neurological symptom terms by manufacturer.

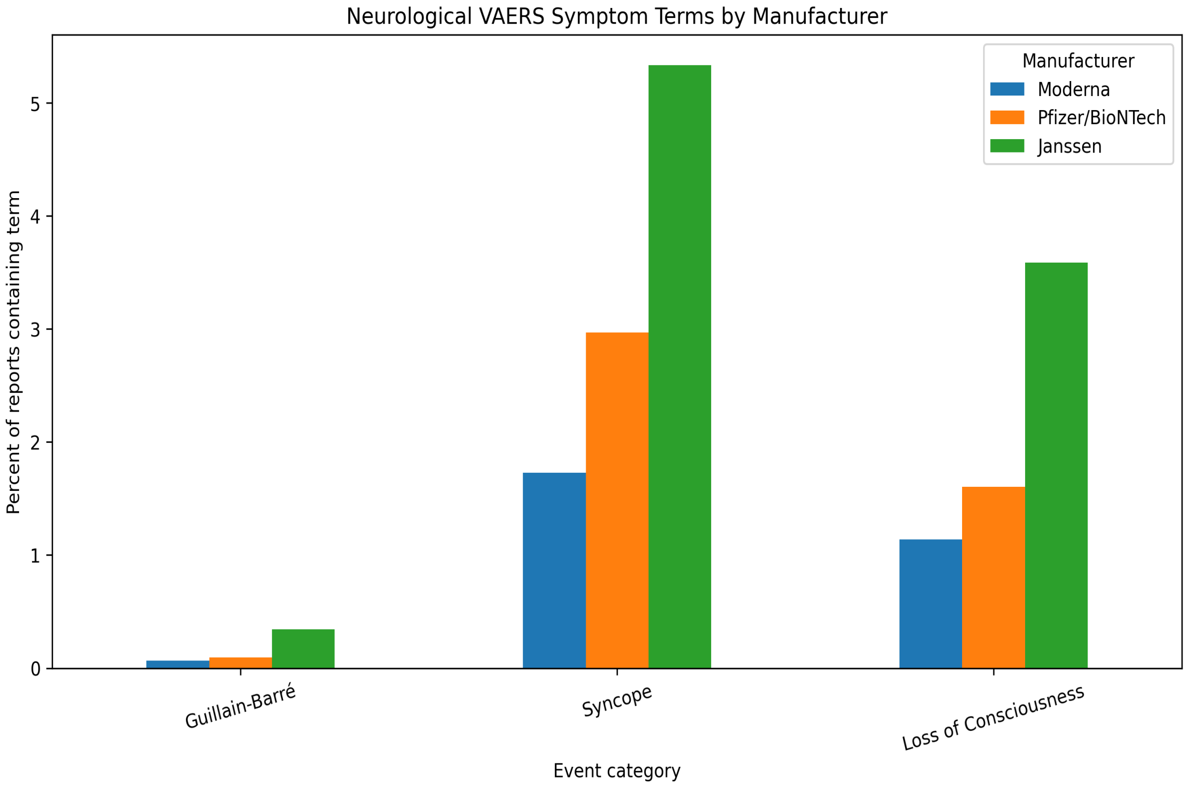
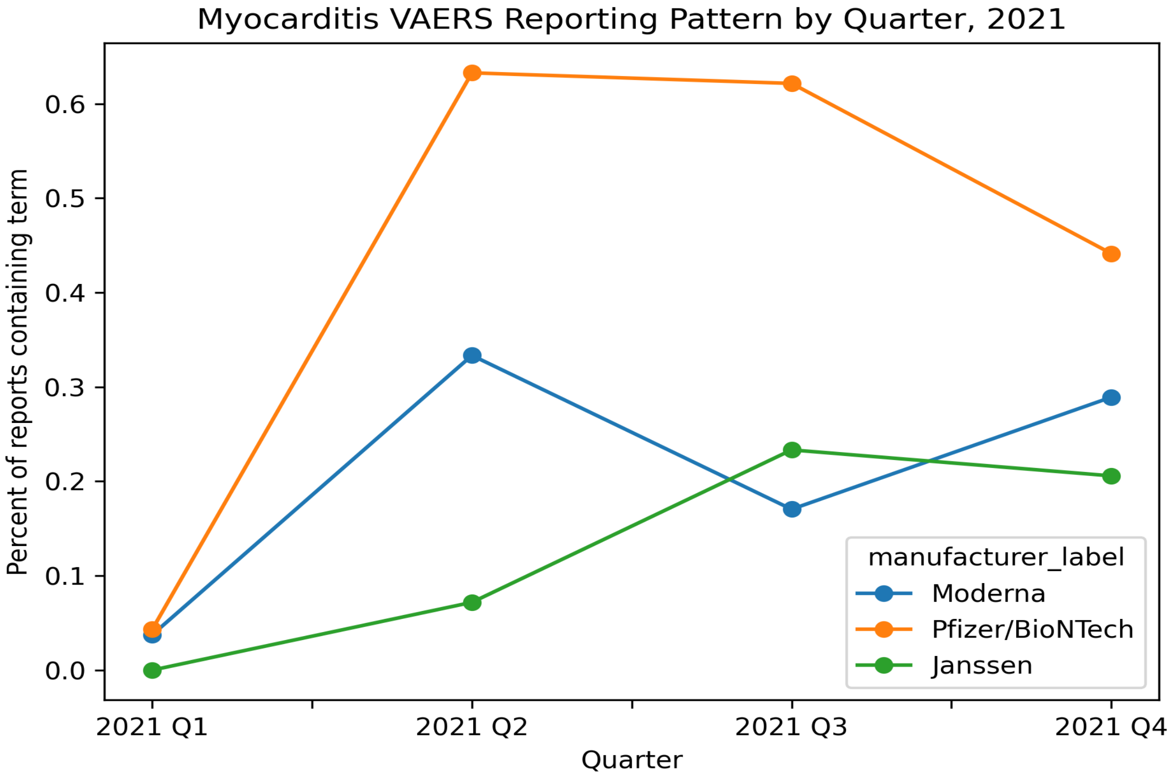


Figure 5. Myocarditis reporting patterns by quarter during 2021.



Limitations

VAERS is a passive surveillance system and does not establish causation. Reporting frequency should not be interpreted as incidence. This analysis did not include administered-dose denominator data and therefore cannot estimate true risk. Observed associations represent reporting patterns within the VAERS dataset.

Future Research

Priority areas include: • Manufacturer-specific denominator data • Age-specific denominator data • Vaccine Safety Datalink comparisons • FDA Sentinel comparisons • Medicare dataset comparisons • Independent replication of findings